Hospital ERP SaaS — Business Requirements (BRD)

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The product must give every subscribing hospital one trustworthy patient identity, correct reconciled billing and departmental states that persist exactly as committed, on one product where each hospital is strictly isolated. Whole-hospital scope is CONFIRMED (C-001). The first production release is OPEN (Q-001); RP-A Outpatient & Diagnostics is PROPOSED as the default.

Draft for review, version 0.4.0. Owner: product owner. Reviewers: pilot hospital management, finance lead, clinical lead. Screens, data and technical design live in the SRS, Database Design and Architecture docs. Where a business rule is undecided, this BRD points to the open question (Q-xxx) instead of choosing.

Status words: **CONFIRMED** = decided by the owner · **PROPOSED** = a recommendation awaiting approval · **OPEN** = undecided · **DEFERRED** = deliberately later · **SOURCE-DERIVED** = taken from the supplied hospital documents · **GAP** = named but not yet specified.

1. Business problem

Staff at the source hospital lose money, time and trust to a legacy HMS whose states do not persist. The supplied documents (S1–S7) describe one hospital, Sondhany Medical (spelling per CX-001), where staff work across reception, cashier desks, doctors' rooms, laboratories, wards, pharmacies and stores. Their reported problems fall into five groups:

1. **Money that does not add up.**
   - Bill totals differ between detail and summary views (SRC-B05).
   - OT and EMR charges are posted twice (SRC-B06).
   - The payment method is not stored as actually received (SRC-B04).
   - Returned medicines are not reflected in the bill (SRC-B03).
2. **States that do not persist.**
   - Lab reports revert to "Pending" after viewing, printing or pressing Back (SRC-L12–L15).
   - Discharge success is shown on screen but not saved (SRC-I05).
   - Bed occupancy disagrees between IPD, Daycare and the nurse station (SRC-I03).
3. **Lost or invisible work.**
   - Post-discharge medicine returns disappear (SRC-N01).

- Return slips are not visible, which causes pharmacy–nursing disputes (SRC-N11).
- Editing one indent quantity deletes the whole indent (SRC-N09).

4. **Slow, error-prone front-line operation.**
   - Pages take 1–1.5 minutes to load, spinners go stale, and keyboard flow is poor (SRC-B01).
   - Phone search fails on formatting (SRC-P02).
   - Bed labels fail to print on the first attempt (SRC-N12).
   - Sessions hang and accounts lock without explanation (SRC-N02, SRC-N10).
5. **Configuration that cannot be trusted.**
   - Ranges and methods do not update from analyzer attributes (SRC-L08).
   - Analyzer results go missing (SRC-L07).
   - Report formats drift (SRC-L02, SRC-L05).

Beyond this hospital, the vendor wants **one maintained product** that many independent hospitals subscribe to (C-001). Each hospital gets its own configuration and is strictly isolated from the others (C-029), instead of separate customized installations.

2. Product objectives

Nine objectives define success; BO-01 to BO-03 carry the core money and state-integrity promises.

ID	Objective	Primary pain addressed	Linked decisions
BO-01	**One trustworthy patient identity and visit context** across all departments. Duplicate registrations are visible and never silently merged.	Wrong-patient risk; inconsistent data across departments (SRC-P01)	C-005..C-007, identity agreements
BO-02	**Correct, reconciled patient billing**: a separate doctor-fee invoice and a combined service invoice; payment before service under configured policy; the actual payment method recorded; no duplicate postings; detail equals summary.	Money that does not add up	C-008..C-011, C-017, C-029

ID	Objective	Primary pain addressed	Linked decisions
BO-03	**Departmental states that persist exactly as committed**: lab statuses, discharge steps, bed occupancy, medicine returns.	States that do not persist	C-014..C-016, C-019, C-029
BO-04	**Fast, keyboard-efficient front-line work** at reception, the cashier desk, sample collection and order entry.	Slow pages, poor keyboard flow	None yet (targets PROPOSED in §9)
BO-05	**Traceability and safe correction.** Every sensitive action is attributable; corrections never destroy evidence.	Lost work, disputes	C-029
BO-06	**Multi-hospital SaaS operation.** Onboard independent hospitals on one product with per-hospital configuration and no cross-hospital data exposure.	Vendor business model	C-001, C-024, C-029
BO-07	**Operable and recoverable on VPS hosting**: monitored, backed up, restorable, supportable.	Hospital continuity	C-026
BO-08	**Bilingual (Bangla/English) operation and correct printing** on the hospital's real A4/A5 and label printers.	Printing and label errors	C-025
BO-09	**Whole-hospital coverage over time**, delivered in dependency order without advertising incomplete modules as complete.	Breadth of hospital operations	C-001, P-005

3. Stakeholders and staff roles

The hospital buys and configures the product; the vendor operates it without routine access to patient data.

3.1 Customers and external stakeholders

Stakeholder	Interest	Notes
Customer hospital (tenant) management	Revenue integrity, operational control, reports, staff accountability	Buys the subscription. Approves the hospital's configuration and policies.
Patients and their proxies	Correct care, correct bills, timely reports, privacy	Portal access is a later, separately designed surface (Q-033).
Referring doctors and B2B diagnostic clients	Report delivery, statements	SOURCE-DERIVED (SRC-C01). Policies OPEN (Q-031, Q-032).
SaaS vendor	One maintainable product, onboarding efficiency, reliable operations	Operates the platform (AS-010). Vendor operators have no routine patient-data access.
Device and payment partners	Analyzer, printer and payment-provider integration	Discovery OPEN (Q-045..Q-047).

3.2 Hospital staff roles (users)

Role names are PROPOSED default templates; each hospital confirms who does what (Q-003). The detailed permission catalogue is in the SRS, Overview §8.

Role	Main work in the product
Receptionist / front desk	Search and register patients; book, reschedule and cancel appointments; check in; create walk-in and direct-diagnostic visits; print tokens.
Cashier	Record payments against selected invoices; print receipts; open and close the cashier session.
Billing supervisor	Approve discounts, refunds and corrections within limits (rules OPEN).
Doctor (OPD/IPD)	Worklist; consultation notes; orders; prescriptions; admission requests; medical discharge.
Nurse	Ward worklist; observations (scope OPEN); indents and medicine returns; bed-related actions where permitted.

Role	Main work in the product
Lab technologist (operator)	Sample collection, receipt, rejection; result entry.
Lab authority (pathologist / senior)	Verification, release, amendment (mapping OPEN, Q-009).
Radiology staff / radiologist	Schedules, exams, reports (module later).
Pharmacist / pharmacy staff	Dispensing, accepting returns, POS (later).
Store keeper / procurement	Stock receipt, transfers, indents, purchase orders (later).
OT / emergency / Daycare staff	Module-specific operations (later).
Finance officer	Collection reconciliation, day-close review, finance reports.
HR officer	Employee records (payroll later, Q-030).
Hospital administrator	Users, roles, configuration, price lists, templates.
Management viewer	Dashboards and reports, read-only.
Vendor operator (not hospital staff)	Tenant provisioning, entitlement flags, service health. Patient data only through a customer-approved support grant.

4. Scope

The whole hospital is in scope (CONFIRMED, C-001), but only some areas are specified well enough to build. The status below describes the *specification*, not whether the product has the capability.

- **SPECIFIED**: detailed enough to build once its blocking questions are answered.
- **PARTIAL**: core flow specified; important rules OPEN.
- **GAP**: named capability with insufficient specification.

4.1 Whole-product scope

Capability area	Specification status	SRS section
SaaS tenancy, users, roles, audit, configuration, localization	SPECIFIED (policy values OPEN)	C01 Platform, access & audit

Capability area	Specification status	SRS section
Patient registration, practitioners, scheduling, check-in, visits	SPECIFIED (MRN format, required fields OPEN)	C02 Patients & scheduling
OPD consultation, orders, prescriptions	PARTIAL (clinical form content OPEN)	C03 OPD, clinical & orders
Patient billing and cashier payments	PARTIAL (deposits, discounts, refunds, rounding OPEN)	C04 Billing & payments
Laboratory (LIS)	PARTIAL (reviewer mapping, formulas OPEN)	C05 Laboratory
IPD admission, beds, transfers, discharge	PARTIAL (bed rent, discharge with dues OPEN)	C06 IPD, beds & discharge
Nursing station	PARTIAL (MAR scope, return sequence OPEN)	C07 Nursing
Pharmacy	PARTIAL (substitution, units, batch rules OPEN)	C08 Pharmacy
Inventory, procurement, reagents	PARTIAL / GAP	C09 Inventory, procurement & reagents
Emergency and Daycare	GAP (triage, payment exceptions, Daycare definition OPEN)	C10 Emergency & Daycare
Radiology and OT	GAP	C11 Radiology & OT
Finance (ledger) and HR/attendance/payroll	**GAP** (explicitly thin in sources)	C12 Finance, HR & payroll
Reporting, printing, patient portal, B2B, referrals	PARTIAL (printing SPECIFIED; portal/B2B/referral GAP)	C13 Reporting, printing, portal & B2B
Integrations and devices	GAP (partner details unknown)	C14 Integrations & devices

4.2 Explicit exclusions and things not assumed

Item	Status	Reason
Autonomous clinical AI (diagnosis, triage, dosing, result interpretation)	Excluded (C-027)	AI helps develop the software only.
Government-specific or statutory requirements, national interfaces, mandated reports	DEFERRED (DEF-001)	Owner request. **No compliance claim is made.**
Multi-facility group features	DEFERRED (DEF-002)	One hospital per customer now (C-024).
Offline mode	DEFERRED (DEF-003)	Paper downtime procedure instead.
Full PACS	DEFERRED (DEF-004)	Partner-dependent.
Blood bank/transfusion, dialysis, dietary/kitchen, ambulance, asset management, insurance claims	**Not in scope until decided**	v0.3 module scope limits. They must not be advertised.
Tax/VAT on patient invoices	Not modeled (AS-014)	Tax rules are deferred with government topics.
SaaS subscription billing automation	DEFERRED (DEF-006)	Commercial decisions OPEN.

5. SaaS customer and facility model

One customer hospital is one tenant with exactly one facility for now; the model is CONFIRMED (C-001, C-024) and its mechanisms are PROPOSED (P-002..P-004).

- A **tenant** is one customer hospital organization. It initially has exactly **one facility** (C-024). The model keeps the facility level so groups can be enabled later (DEF-002).
- Each tenant has its own users and memberships, roles (copied from templates), price lists, payment-eligibility policies, numbering patterns, print templates, lab configuration and language defaults.
- **One product, many configurations.** No customer-specific code branches (P-004). A hospital-specific behavior is either a configurable policy listed in SRS Overview §6, or a product change that applies to everyone.

- **Patient identity belongs to the tenant.** The same person at two hospitals is two unrelated patients. The same MRN may exist in two tenants without any link.
- **Entitlements are not permissions.** Purchasing the LIS module (entitlement) does not let anyone release a result; users still need a permission (FR-SAA-004).
- **Vendor operations are separate.** Vendor operators provision tenants and view service health. They access patient data only through a time-limited, customer-approved, audited support grant (FR-SUP-001).
- **Commercial lifecycle** (trial, active, suspended, terminated) is OPEN (Q-052). Its rules must protect active patient care (FR-SAA-007).

Onboarding outcome (BW-14). A new hospital is provisioned, configured and verified without vendor database edits. A retried provisioning step does not create a duplicate tenant.

6. Current pain points

Fifteen pain points come from the supplied sources; each maps to at least one objective.

#	Pain point	Source	Business impact	Objective
PP-01	Page loads and order entry take 1–1.5 minutes; spinners go stale; keyboard flow is poor; required doctor/MO fields missing	SRC-B01	Queues at reception and cashier; staff workarounds	BO-04
PP-02	Detail vs summary totals disagree; fraction adjustments are implicit; service charge is computed inconsistently	SRC-B05	Revenue disputes, audit failure	BO-02
PP-03	Duplicate EMR/IPD and OT charges	SRC-B06	Overbilling, refunds, reputation	BO-02
PP-04	Payment method (cash/card/bKash) not stored as received; corrections not traceable	SRC-B04	Cash reconciliation failures	BO-02, BO-05

#	Pain point	Source	Business impact	Objective
PP-05	Accepted medicine returns not reflected in net charges; medicine and non-medicine subtotals mixed	SRC-B03, SRC-N01	Patient overcharged; disputes	BO-02, BO-03
PP-06	Lab reports revert to Pending after Back, Print or Customize; processing does not persist status	SRC-L12–L15	Reports re-verified or wrongly delivered; clinical risk	BO-03
PP-07	Analyzer (EXL-200 Blood Urea) results missing; immunology attributes do not update range/method	SRC-L07, SRC-L08	Manual re-entry, wrong reference ranges	BO-03 (integration later)
PP-08	Lab printing: formats drift, fonts change after save, advice auto-bolded, biochemistry serials wrong, room/bed missing	SRC-L02, L04–L06, L10	Rework, wrong-ward delivery	BO-08
PP-09	Bed occupancy inconsistent across IPD, Daycare and nurse station; dummy beds confusing; extra beds untagged	SRC-I01–I03	Double allocation, lost patients	BO-03
PP-10	Discharge shows success but is not saved; discharged patients not clearly identified	SRC-I05, SRC-N06	Beds blocked; care confusion	BO-03
PP-11	Indent quantity edit deletes the whole indent; one-by-one entry; return slips invisible	SRC-N08, N09, N11	Nursing time lost; pharmacy disputes	BO-03, BO-05
PP-12	Sessions hang; re-login fails; account-lock reasons not visible	SRC-N02, SRC-N10	Nurses locked out during shifts	BO-04, BO-07
PP-13	Phone search fails with spaces/hyphens; duplicate doctors by name	SRC-P02, SRC-P03	Duplicate patients, wrong doctor	BO-01

#	Pain point	Source	Business impact	Objective
PP-14	Bed number does not print on first attempt	SRC-N12	Wrong labels on wards	BO-08
PP-15	Mistaken X-ray order cannot be cancelled cleanly	SRC-N07	Wrong charges, manual fixes	BO-02, BO-05

7. Business workflows and outcomes

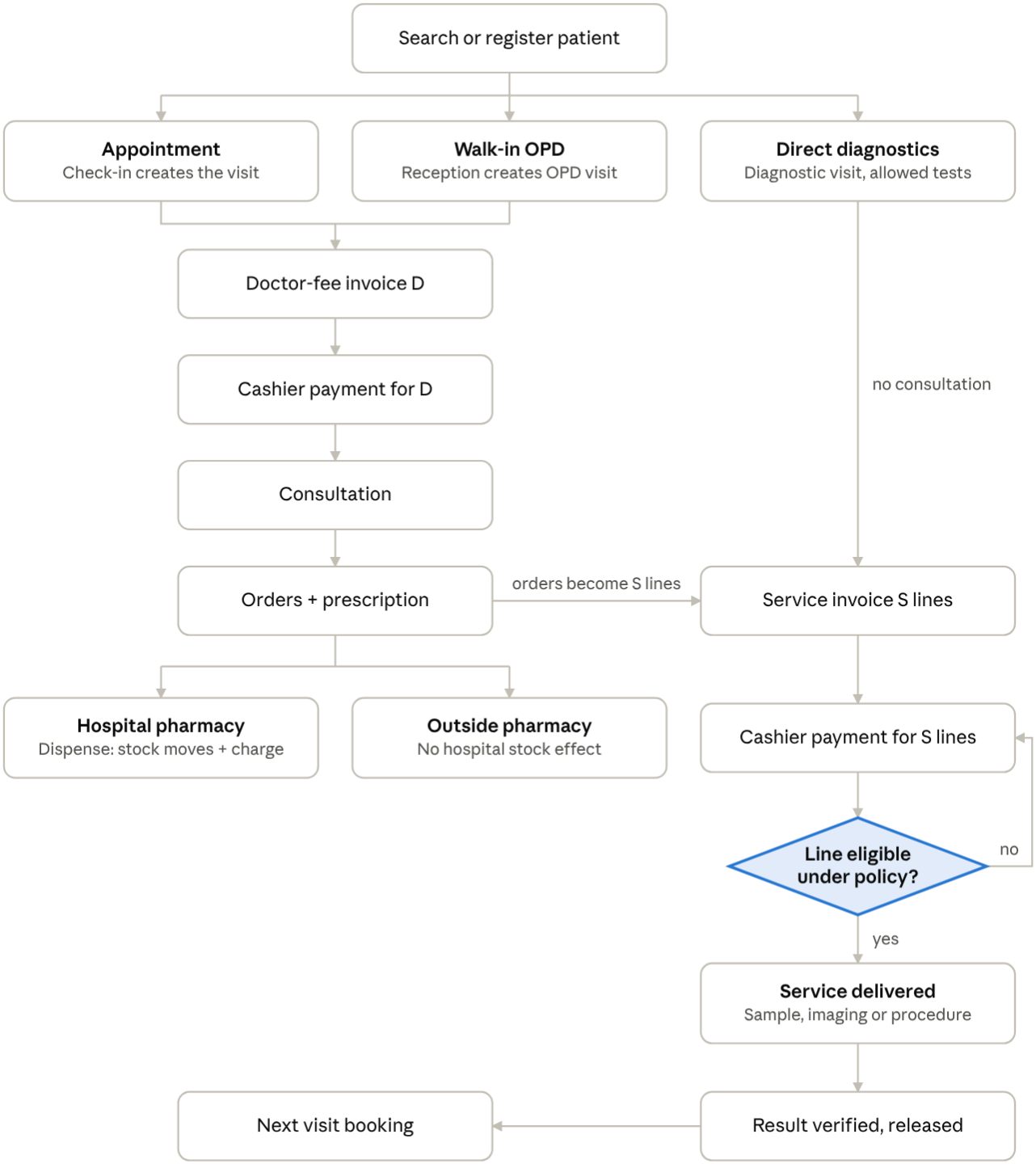
Sixteen workflows each name the business outcome they must reliably produce; actors, states and paths are in the SRS.

ID	Workflow	Business outcome	Status	SRS
BW-01	**OPD consultation journey**: search/register → check-in or walk-in → doctor-fee invoice → cashier payment → consultation → orders/prescription → service invoice → payment → services → results → next visit	Patient is served in order; doctor fee and service charges are paid and reported separately; nothing starts unpaid unless policy allows.	CONFIRMED (C-005, C-008, C-009, C-011)	C02, C03, C04
BW-02	**Direct diagnostic visit**: reception creates a diagnostic visit with directly orderable tests → service invoice → payment → sample → result → release → delivery	A diagnostic-only patient never needs a consultation; restricted tests cannot be ordered at the desk.	CONFIRMED (C-006)	C02, C03, C05
BW-03	**Appointment and next visit**: book, reschedule, cancel, check in	Bookings never count as attendance; history of changes is kept.	CONFIRMED (C-007, C-018)	C02

ID	Workflow	Business outcome	Status	SRS
BW-04	**Laboratory order to release**: eligibility → label/collect → (dispatch) → receive or reject/recollect → enter → verify → release → deliver; amendments versioned	Released reports never regress; every release is attributable; rejected samples are traceable.	CONFIRMED lifecycle (C-019); actor mapping OPEN (Q-009)	C05
BW-05	**Inpatient stay**: admission (OPD / emergency / planned) → wait for bed or allocate → transfers → charges and deposits on running account → discharge steps → final bill → departure → cleaning → bed available	Occupancy always matches reality; the bed is not reused before cleaning; charges post once; the final bill accounts for deposits exactly once.	CONFIRMED (C-013..C-017); settlement OPEN (Q-006, Q-007)	C06, C04
BW-06	**Nursing indent and return**: batch indent → pharmacy issue → batch return → pharmacy acceptance → credit	Both sides see the same return record; quantities and credits reconcile.	SOURCE-DERIVED; sequence OPEN (Q-018)	C07, C08
BW-07	**Prescription fulfilment**: hospital pharmacy dispense (stock + bill) or outside pharmacy (no stock effect)	Hospital stock moves only on actual dispensing.	CONFIRMED (C-012)	C03, C08
BW-08	**Emergency arrival**: known or temporary identity → staff-assigned priority → orders → admit/refer/discharge	Care is never blocked by unknown identity; reconciliation is authorized and traceable.	CONFIRMED identity rules; payment exceptions OPEN (Q-005)	C10
BW-09	**Daycare stay**	Same-day stays never show an occupied bed as available.	SOURCE-DERIVED; definition OPEN (Q-026)	C10
BW-10	**Radiology exam and report**	Report tied to the right order and version.	SOURCE-DERIVED (Q-028)	C11

ID	Workflow	Business outcome	Status	SRS
BW-11	**OT booking to clearance**	OT stages and charges post once to the patient account.	SOURCE-DERIVED (Q-027)	C11
BW-12	**Procurement and stock**: requisition → PO → approval → GRN → stock → transfer/issue	Stock quantities follow movements only; receipts are never double counted.	SOURCE-DERIVED (Q-063, Q-064)	C09
BW-13	**Cashier session and day close**	Each cashier's collections by method reconcile with receipts and with the collection report.	SOURCE-DERIVED (SRC-B07)	C04, C13
BW-14	**Hospital onboarding**: provision tenant → facility → admin → entitlements → masters/prices/templates → verify → activate	A hospital can go live without vendor database edits; isolation verified.	CONFIRMED direction (C-001)	C01
BW-15	**Finance reconciliation and HR/payroll**	To be defined.	**GAP** (Q-029, Q-030)	C12
BW-16	**Reports, portal and B2B delivery**	Authorized, reconciled reporting; delivery only of released reports.	PARTIAL	C13

Doctor fee and services are paid separately; a service starts only when eligible



Outpatient and direct-diagnostic journey (BW-01, BW-02, BW-07)

Paying the doctor fee D never pays any service line S; each S line is checked against the hospital's configured payment policy before the service starts (C-008..C-011). Hospital stock moves only when the hospital pharmacy dispenses (C-012).

8. Release options

PROPOSED: plan for RP-A Outpatient & Diagnostics. The first production release is not approved (Q-001). Every option includes the foundation: tenancy, access, audit, configuration, bilingual UI, printing baseline, backups and CI. Each option assumes its blocking questions are answered. A department must not be marketed as complete when only part of it is delivered (v0.3 PRD rule).

Option	Scope	Suits a pilot that…	Blocking questions	Main risks
RP-A: Outpatient & Diagnostics (recommended default)	Foundation; patients & scheduling; walk-in, appointment and direct diagnostic visits; doctor-fee invoice + cashier; consultation notes + prescription print (outside fulfilment only); orders; service invoice + payment gate (full payment); LIS with manual entry, verification, release, A4 reports, specimen labels; cashier day close; core reports	…can run IPD, pharmacy and stores on the legacy system or paper for a period	Q-003, Q-009, Q-017, Q-020, Q-022, Q-024, Q-034, Q-035, Q-037, Q-038, Q-039, Q-046, Q-050, Q-051. Q-004 answered **or** deposits excluded.	Two systems in parallel during the transition; IPD patients' OPD history split
RP-B: Outpatient, Diagnostics & Inpatient core	RP-A + admission, bed board, allocation/transfer/cleaning, running account with deposits, final bill, nursing worklist with batch indent/return, minimum pharmacy dispensing with batch/expiry and returns	…cannot run IPD on paper or legacy, as the Sondhany sources suggest	RP-A list + Q-004 (required), Q-006, Q-007 (or manual bed charges), Q-008, Q-014, Q-015, Q-016, Q-018, Q-040, Q-063	Largest scope; IPD money rules unresolved today; longest path to first value

Option	Scope	Suits a pilot that…	Blocking questions	Main risks
RP-C: Diagnostic centre first	Foundation; patients; direct diagnostic visits; service invoice + cashier; LIS; report delivery; day close	…is a diagnostic centre or wants the lab first	Q-003, Q-009, Q-020, Q-022, Q-034, Q-035, Q-039, Q-046, Q-050, Q-051	Does not exercise the doctor-fee invoice; OPD follows separately

Why RP-A is the PROPOSED default:

1. It delivers the confirmed core money model (C-008..C-011) and fixes the lab problems with the strongest source evidence (PP-02..PP-08).
2. Its blocking questions can be answered by the front desk, cashier, finance and lab leads, without the unresolved IPD settlement and bed-rent rules (Q-006, Q-007).
3. It builds every foundation RP-B needs, so choosing RP-B later costs no rework.

Choose **RP-B** if the pilot profile (Q-042) shows IPD cannot stay on the legacy system; Q-006 and Q-007 then become priority P1, or manual bed charges are accepted as an interim measure. Choose **RP-C** if the first customer is diagnostics-led. The implementation phases for each option are in the Implementation Plan (G §2) of the full documentation pack.

9. Success measures

Eleven measures make success checkable; every numeric target is PROPOSED, not approved or promised. Baselines must be measured at the pilot before go-live (for example, current registration time).

ID	Measure	How measured	Target	Status
SM-01	Cross-tenant data exposure	Automated isolation suite on every release; incident log	Zero exposures; suite 100% pass	Requirement CONFIRMED (C-029); measurement PROPOSED
SM-02	Daily collection reconciliation	Cashier session totals by method = sum of receipts = collection report, per day	100% of days reconcile without database edits	PROPOSED

ID	Measure	How measured	Target	Status
SM-03	Duplicate financial postings from retries or repeated events	Duplicate-detection query on `charge.source_ref` and payment idempotency keys	Zero	PROPOSED
SM-04	Front-desk task time	Timed observation: returning-patient search → check-in → doctor invoice issued	Median below the measured legacy baseline; numeric target set after baseline	PROPOSED
SM-05	Interactive response time	API p95 for search, check-in and payment under the agreed load (Capacity Plan M §7)	e.g. p95 ⩽ 800 ms at pilot peak (to be confirmed)	PROPOSED
SM-06	Lab status regressions	Regression suite AT-LAB-*; production audit of RELEASED → non-released transitions without amendment	Zero	PROPOSED
SM-07	Bed board accuracy	Spot-check physical census vs bed board	100% match at each check	PROPOSED (RP-B only)
SM-08	Duplicate patient registrations	Share of new registrations later flagged as duplicates	Trend down from baseline	PROPOSED
SM-09	Recoverability	Timed restore drill reaches a consistent state	Within the RTO agreed in Q-043	Measure CONFIRMED by C-029; value OPEN
SM-10	UAT acceptance	Department UAT pass rate at release gate RG-3 (Manual/UAT Plan I)	All P1 cases pass; no open Severity 1/2 defects	PROPOSED

ID	Measure	How measured	Target	Status
SM-11	Staff-reported blockers in the first weeks after go-live	Support tickets by category	Downward trend; no unresolved "cannot complete work" ticket older than one business day	PROPOSED

10. Dependencies

The release depends first on business answers and a pilot hospital profile, then on hosting, tooling and partners.

Type	Dependency	Needed for	Owner
Business decisions	Open questions per release option (§8)	Ready tasks	Product owner + responders named in the open-questions register
Pilot hospital	Profile (Q-042), staff for UAT, sample documents (receipts, report formats, label sizes)	RG-3	Product owner
Devices	Printers, label printers, scanners at the pilot (Q-046)	Print UAT	Pilot IT
Data	Legacy migration scope (Q-048)	Cutover	Product owner
Hosting	VPS provider and region (Q-044); off-site backup storage	RG-1 staging, RG-4	DevOps
Tooling	GitHub plan (Q-055); SonarQube host (Q-056)	CI	DevOps
Language	Native-speaker review of Bangla labels and print templates (Q-050)	RG-3	Product owner

Type	Dependency	Needed for	Owner
Partners	Analyzer vendor (Q-047), payment provider (Q-045)	Integration slices	Product owner

11. Business acceptance criteria

These twelve criteria apply to whichever release option is approved. "Accepted" means the named business owner signs off on evidence from the Manual/UAT Plan (I), not merely that automated tests pass.

ID	Criterion	Evidence	Signs off
BAC-01	A returning patient can be found by name, MRN or phone in any common format. A new registration warns about likely duplicates and never auto-merges.	UAT-REC-001..004	Front desk lead
BAC-02	For a visit with doctor fee D and services S, paying D leaves S's balance unchanged, and receipts and reports show D and S separately.	UAT-CSH-002; AT-BIL-001	Finance lead
BAC-03	A service cannot start unless its line is eligible under the configured policy. The UI explains why.	UAT-CSH-004, UAT-LAB-001	Finance + lab lead
BAC-04	Each cashier's session closes with totals by method that match receipts and the daily collection report.	UAT-CSH-010	Finance lead
BAC-05	A released lab report stays released through view, print and back navigation. Corrections create a visible amendment.	UAT-LAB-008..010	Pathologist

ID	Criterion	Evidence	Signs off
BAC-06	A rejected sample keeps its reason, and the recollected sample is linked to it.	UAT-LAB-005	Lab manager
BAC-07	Printed receipts, reports and labels are legible and correctly laid out in Bangla and English on the pilot's printers.	UAT-PRN-*	Pilot IT + department leads
BAC-08	A staff member from one tenant can never see or change another tenant's data through any screen, report, export or file.	AT-ISO suite + UAT-ADM-006	Product owner
BAC-09	A restore drill on staging returns to a consistent state, and staff can continue the core workflow.	Restore record (VPS Operations runbook RB-07)	Product owner + DevOps
BAC-10	*(RP-B only)* An admission can wait for a bed. Transfers keep history. A discharged bed cannot be allocated before cleaning. The final bill counts deposits exactly once.	UAT-IPD-*	IPD manager + finance
BAC-11	Every sensitive action shown in the audit view identifies who, when, what record and why (where a reason is required).	UAT-ADM-004	Hospital administrator
BAC-12	Staff can work the full journey of the release in their chosen language (Bangla or English) and switch without losing work.	UAT-I18N-*	Front desk + native reviewer

12. Business risks

The six business risks below matter most; the full register (R-01..R-18) is in the Traceability & Readiness document (N §6).

- A wrong patient is selected (R-01).
- A doctor-fee payment unlocks unpaid tests (R-02).
- Cross-tenant data exposure (R-03).
- Duplicate financial postings (R-04).
- An AI coding agent invents unresolved policy (R-10).
- The release scope is chosen without the pilot profile (R-12, new in v0.4).